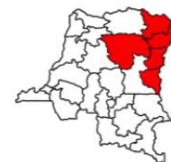




EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°110/MVEBDB/01/09/2026

Reporting date: September 1, 2026

Publication date: September 2, 2026



HIGHLIGHTS (last 24 hours)

In the last 24 hours, 64 new confirmed cases of Ebola Virus Disease have been reported, including 25 community deaths. (EVD) were recorded in the provinces of Ituri (39 cases), North Kivu (20 cases), Haut Uélé (4) and (1) in Tshopo. In addition, 7 intra-CTE deaths were recorded. No new health zones affected in the last 24 hours, with overall notification down (-25.6%)

ACCUMULATIVE CASE 6,250	ACCUMULATIVE DEATH CONFIRMED 3039	LETHALITY 48.6%	PATIENTS IN ISOLATION/CTE 869	ACCUMULATIVE HEALED 1,439	TRACKING RATES CONTACTS (Of the Day) 89.0%
--	--	----------------------------------	--	--	---



Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



60 out of 151 health zones affected (39.7%)



Summary of key points from the last 24 hours

As of September 1, 2026, **64 new confirmed cases**, including **25 community deaths** from Ebola Virus Disease (EVD), were reported in the provinces of **Ituri (39 cases), North Kivu (20), Haut Uélé (4) and (1) in Tshopo**.

Meanwhile, **7 intra-CTE deaths** were recorded, including 6 in Ituri and 1 in Haut Uélé, bringing the number of deaths for the day to **32**.

However, **30 patients** were declared cured after obtaining two negative results for Ebola virus disease (27 in Ituri, 2 in North Kivu and 1 in Tshopo).

Cumulatively, the DRC has **6,250 confirmed cases, including 3,039 deaths**, representing a case fatality rate of **48.6 %**. Ituri province remains the epicenter, accounting for **81.7%** of the country's confirmed cases and **60.9%** of new confirmed cases reported in the last 24 hours.

Since the start of the epidemic in the DRC, **60 out of 151 health zones (39.7%)** remain affected across the six affected provinces. Ituri has 28 affected health zones out of 36, followed by North Kivu (15/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 1, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	39	5104	2,324	45.5%	28/36 (77.8%)
North Kivu	20	888	601	67.7%	15/34 (44.1%)
Haut-Uélé	4	231	101	43.7%	6/13 (46.1%)
Tshopo	1	20	9	45.0%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	64	6,250	3039	48.6%	60/151 (39.7%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 1, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5104	2324	45.5%	39	13	6	19
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	165	36	21.8%				0
Boga	2	2	100.0%				0
Bunia	1413	442	31.3%	6	1		1
Damascus	28	9	32.1%				0
Drodro	14	7	50.0%				0
Fataki	77	30	39.0%	1	0		0
Gethy	6	2	33.3%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	92	62	67.4%	5	3		3
Lita	224	121	54.0%	4	2		2
Logo	11	5	45.5%				0
Lolwa	22	5	22.7%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	36	21	58.3%				0
Mangala	263	134	51.0%	6	1		1
Mongbwalu	615	287	46.7%	3	1		1
Nia-Nia	216	113	52.3%				0
Nizi	670	264	39.4%	6	3		3
Nyankunde	124	35	28.2%				0
Rimba	11	4	36.4%	1	0		0

Rwampara	964	341	35.4%	7	2		2
Tchomia	57	25	43.9%				0
To ventilate	N / A	345	N / A	N / A	N / A	6	6
North Kivu	888	601	67.7%	20	12	0	12
Blessed	146	105	71.9%	5	3		3
Good	1	1	100.0%				0
Butembo	180	134	74.4%	6	3		3
Goma	1	0	0.0%				0
Kalunguta	16	7	43.8%				0
Katwa	408	270	66.2%	4	2		2
Kyondo	16	12	75.0%				0
Lubero	3	3	100.0%	1	1		1
Mabalako	6	5	83.3%	1	1		1
Manguredjipa	2	1	50.0%	1	1		1
Masereka	11	5	45.5%	2	1		1
Musienene	83	47	56.6%				0
Mutwanga	2	2	100.0%				0
Oicha	7	6	85.7%				0
Vuhovi	6	3	50.0%				0
Haut-Uélé	231	101	43.7%	4	0	1	1
Boma	27	14	51.9%				0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	85	34	40.0%	4	0	1	1
Pawa	37	22	59.5%				0
Rungu	1	0	0.0%				0
Wamba	78	30	38.5%				0
Tshopo	20	9	45.0%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	9	5	55.6%				0
Mangobo	3	2	66.7%	1	0		0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6250	3039	48.6%	64	25	7	32

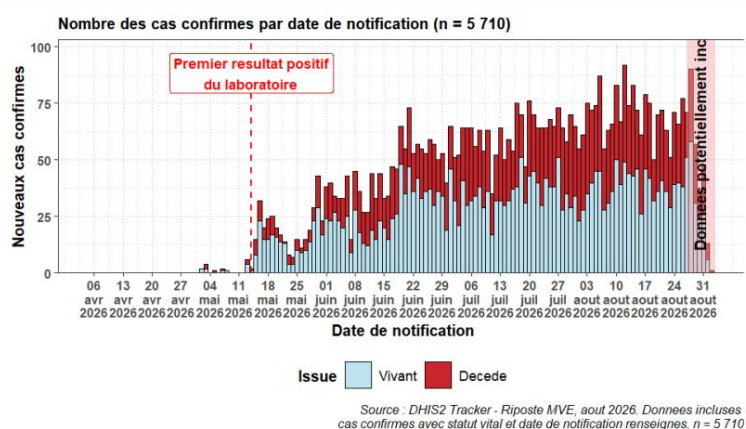
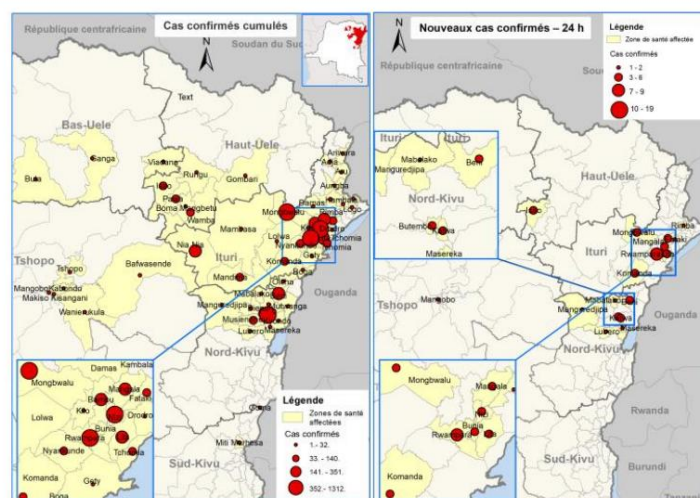
*These are the confirmed deaths that occurred in the various CTES in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTES of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 1, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	12	2	14	12	1	0	0	13	10
Bas Uélé	25	0	25	4	0	21	0	4	0
Ituri	803	74	877	142	74	626	0	216	125
North Kivu	608	42	650	70	38	501	4	108	50
South Kivu	ND	ND	0	ND	ND	ND	ND	ND	ND
Tshopo	138	1	139	6	1	132	0	7	1
Total General	1586	119	1,705	234	114	1280	4	348	186

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Over the past 24 hours, 1705 alerts have been recorded in the 6 provinces. Of the 1,632 alerts (95.7%), 348 were verified. These were validated as suspected cases. All of these were investigated (100%), and 186 were transferred to CT/CTE (Table 3).

The proportion of contacts followed up in the last 24 hours is 89.0% (16,719 seen out of 18,784 to be followed up), above the threshold of 85%, compared to the previous day (88.0%); 100% in Tshopo (350/350), 90.1% in North Kivu (7,468/8,293), 89.7% in Ituri (7,115/7,932), 84.6% in Haut Uélé (1,562/1,846) and 61.7% in Bas Uélé (224/363).

1.1.3. Challenges

Persistent low contact tracing around confirmed cases in the Lower Province Uélé, linked to a lack of teams dedicated to these activities.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Six (06) live alerts were notified to the PoE/PoC in the last 24 hours, of which five were validated and referred: two in Ituri, at the PoC of Iga-Barrière (ZS Nizi) and at the PoC of Irumu Centre (ZS Nyankunde), both isolated and referred to the CTEs of Bunia and Nyankunde; four in North Kivu of which three were validated, all referred to the dedicated structures.

Table 4. Status of key indicators for PoC/PoE by province, as of September 1, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	96.0% (58/60)	100% (18/18)	55.5% (5/9)	76.47% (13/17)	54.5% (4/11)	80% (12/15)	79.13% (91/115)
% of functional strategic PoE/PoC	9.09% (1/11)			100% (17/17)			9.1% (1/11)
Number of people who have switched to PoE/PoC	145990	88249	6609	3,981	15,539	6091	266,459
% of travelers screened	99.00%	96.83%	98.00%	99.90%	98.30%	98.16%	98.41%
% of travelers who washed the hands	97.90%	96.83%	98.00%	99.30%	96.00%	98.16%	97.61%
% of travelers made aware	99.50%	96.83%	100%	100%	96.60%	98.16%	98.59%
Number of alerts notified	2	4	0	0	0	0	6
Number of validated live alerts (suspected cases)	2	3	0	0	0	0	5
Number of bodies intercepted today; % of suspected	0	0	0	0	0	0	0
cases transferred to CT/CTE	100%	100%	100%	100%	0%	-	100%
% of lifeless bodies swabbed/secured	0%	0%	ND	100%	0%	-	0%
% of suspected cases investigated within 24 hours	100%	100%	100%	ND	0%	-	100%

1.2.2. Challenges

Operationalization of PoE/PoC facilities is suboptimal, and reporting is low in South Kivu (55.5%) and Haut-Uélé (54.5%). In Ituri, only 1 (9.09%) of the 11 strategic PoE/PoC facilities remains fully operational: the Avakubi PoC is non-functional for security reasons, and the Nzebi, Pont Ituri, Makoko 2, and Foner Mambasa PoC facilities are experiencing strikes due to non-payment. In Haut-Uélé, 4 PoC facilities (Dusu, Apodo, PK10 Poko road, and Gossamu) have not reported, and the Magambe PoC has recorded refusals to undergo screening (3%) and handwashing (13%). In South Kivu, isolation facilities are lacking at the Kavimvira, Kalundu Port SNCC, and Baraka PoE facilities.

remains.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 39 new positive results** (26 living and 13 dead) out of 347 new samples received and analyzed (**positivity: 11.2 %**).

- **North Kivu : 20 new positive results** (8 living and 12 dead) out of 110 samples analyzed (**positivity of 18.1%**).
- **Haut-Uélé: 4 new positive results** (3 living and 1 death) out of 26 samples analyzed (**positivity: 15.3%**).
- **Bas-Uélé: 1 sample** received and tested (1 blood sample), came back negative.
- **Tshopo:** Provision of equipment (1 Epson printer, 3 Split 12,000 BTU), binders, pens, markers, duplicator paper, printer cartridges; briefing of morgue and EDS agents on SWAB sampling.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 19 rings were opened out of the 45 identified (42.2%), 113 households decontaminated out of 116 identified (97.4%) and 13 FOSA out of 13 (100%); 77 EDS carried out out of 126 alerts and 65 bodies swabbed (42 failures).
- **In North Kivu**, 10 out of 30 rings were opened (33.3%), 6 out of 21 identified households were decontaminated (28.6%), and 21 out of 28 health facilities were decontaminated (75%); 6 health facility assessments were carried out with an average score of 29%. Furthermore, 51 bodies were swabbed and 59 blood and sanitation assessments (BSAs) were conducted.
- **In Tshopo**, decontamination of 6 premises, 1 latrine and 1 shower of PEKIS (17th case); monitoring and support of 11 ESS in 5 ZS (Makiso-Kisangani, Mangobo, Tshopo, Kabondo and Lubunga) of which 116 providers were briefed on the implementation of IPC measures; decontamination of 6 premises, 1 latrine and 1 shower at PEKIS and implementation of a screening and triage system at the Bafwasende General Referral Hospital.

1.4.2. In Bas-Uélé, 4 out of 4 rings open (100%), 2 out of 2 households identified decontaminated (100%) and 1 out of 1 FOSA (100%); 1 FOSA evaluation in Buta.

1.4.3. Challenges

- Weak implementation of IPC standards: in Ituri, ring opening remains low (42.2%); insufficient decontamination teams in Fataki, Mangala, and Nizi. In North Kivu, only 6 households out of 21 identified were decontaminated (28.6%). In Tshopo, there is a shortage of IPC supplies in health establishments and a breakdown of the incinerator at the Makiso-Kisangani General Referral Hospital.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 112 new admissions to the various dedicated facilities, compared to 87 discharges, including 27 recoveries. Thus, 564 patients are hospitalized in 978 beds, representing an occupancy rate of 57.7%.
- **In North Kivu**, 60 new admissions and 62 discharges including 2 recovered; 232 patients are hospitalized and the overall occupancy rate of beds in CTE/CT is oversaturated (105.5%; 232/220).
- **In South Kivu**, no new admissions were recorded. Eight (8) patients are in isolation for 25 beds, representing an overall occupancy rate of 32.0%.
- **In Haut Uélé**, 11 new admissions were recorded. At the end of the day, 62 patients were admitted, representing an overall occupancy rate of 51.7% (120 beds).
- **In Tshopo**, one new admission was recorded, compared to three discharges, one of whom recovered. Three (3) patients are in isolation out of 25 beds, representing an overall occupancy rate of 12.0%.

1.5.2. Challenges

Low capacity and saturation: in Ituri, the CTEs of Bambu (confirmed) and Nizi (suspected) as well as the CTs of Kigonze and Lolwa (suspected) are saturated; in North Kivu, over-saturation of some CT/CTEs; North Kivu.

1.6. Risk communication and community engagement

1.6.1. Main actions

- **In Ituri**, 12 people were informed about the importance of contact tracing and adherence to response actions; 55 people were sensitized on the importance of IPC actions; 15 people were informed and sensitized on the importance of adherence to IPC actions and regular contact tracing in the Rwampara health zone.
- **In North Kivu**, 14,794 home visits were carried out, reaching 45,772 people; 77 educational talks were conducted with 1,949 people; 3 mass communications (459 people) and 4 briefings (153 people) were conducted; 8 feedback and infodemics were resolved within 48 hours out of 8 received and 150 alerts were notified by community actors; 35 pharmacy owners were briefed in Musienene.
- **In Tshopo**, 1,289 people sensitized (696 women and 593 men); 2 rumors clarified and 2 community feedbacks collected; 302 leaflets and 58 posters distributed; awareness-raising of promoters of private schools in Kisangani and continuation of home visits in the health zones of Kabondo and Lubunga.
- **In Bas-Uélé**, awareness-raising of more than 200 people during the mourning of a positive case in the village of Bobagata (AS Mabodho, ZS Ganga), briefing of 30 press actors in Buta and community dialogue in AS Eganda (ZS Bondo), with the support of FHI360.

1.6.2. Challenges

Persistence of challenges at community level: in Ituri, 17 refusals of isolation at the CTE, 42 bodies not swabé due to community resistance and resistance in Bambu, Mandima and Fataki; in Tshopo, resistance to investigations, sampling, isolation and transfer to the CTE.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 17 people among the new confirmed cases and 159 of the 209 confirmed cases under follow-up (76%) benefited from SMSPS interventions; 53 of the 112 new suspected cases (47.3%); 77 households received support, 41 people with HIV received assistance, and 74 test results were announced, including 17 positive and 14 recovered out of 16 followed in the community (87.5%).
- **In North Kivu**, 28 new confirmed cases and 132 of the 167 suspected cases benefited from SMSPS interventions; 103 of the 123 test results were announced, including 31 positive cases, 58 people receiving support and 45 households receiving assistance. In Tshopo, 1 confirmed case and 2 suspected cases are under follow-up; 3 test results were announced and 2 caregivers received support at the treatment center; 2 recovered individuals have joined the response teams.
- **In Bas-Uélé**, 2 laboratory results were announced, including 1 positive result and 3 PPLs (Personal Protective Equipment) which benefited SMSPS interventions.

1.7.2. Challenges

In Ituri, SMSPS interventions have limited coverage around the latest confirmed cases; coverage is restricted to 6 of the 10 health zones that have reported cases (60.0%). In Tshopo, there are no SMSPS providers in the health zones of Wanie-Rukula and Bafwasende.

1.8. Logistics

1.8.1. Main actions

- **In Tshopo**, monitoring of the progress of the construction of the Madula isolation units and a High-capacity CTE; dispatching of motorcycles to the various pillars; mobility support

PCI, PECH, Communication, PoE/PoC and vaccination pillars; unpacking and stock updates.

- **In Ituri**, delivery of IPC supplies to the Mangala Health Zone and 3,570 kg of food (1,922 packages) to the Tchomia Health Zone, a donation from Save the Children.
- **In**

North Kivu, delivery of IPC supplies to the mitigation and security pillar and a visit mission to the Ebola Treatment Centers (ETCs) in Katwa and Beni with WHO and WFP.

1.8.2. Security

1.8.3. Main actions

- **In North Kivu**, 12 activities to maintain public discipline were carried out, including security support for IPC teams for decontamination in Beni (AS Malepe, Ndindi, Mangote and Ngongolio) and EDS teams in Katwa (AS Kivika, Mukuna and Kangote).

1.8.4. Challenges

Persistent escapes from treatment centers/emergency care facilities: Two escapes of suspected cases were recorded in Mangala (Ituri), with patients leaving the hospital at night. In North Kivu, officials are resisting adherence to health protocols at points of contact (PoCs), and security has been reinforced at the Vulindi PoC (Butembo Health Zone).

1.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.9.1. Main actions.

In Ituri, briefing of 97 people of whom 76 were trained on the prevention of sexual exploitation, abuse and harassment in the Bunia health zone and 21 people in the Nizi health zone; continued with the display of messages and distribution of PSEAH leaflets in the health zones of Bunia, Aru, Rwampara, and Mongbwalu.

1.9.2. Challenges

- Still insufficient coverage of PSEA activities: 25% of the health zones in Ituri (9/36) are aware of the reporting mechanism; lack of mapping of stakeholders; insufficient mass awareness materials; lack of means of transport and computer equipment for the pillar.

KEY MESSAGES

• At the end of this day, a maintenance of performance was observed in epidemiological investigations (**100% of suspected cases investigated**) and a stability on the daily proportion of contact tracing **89.0% vs 88.0% the day before**.

• Continued transmission of EVD, with **64 new confirmed cases**, including **25 deaths community and 7 intra-CTE cases** recorded in the last 24 hours, bringing the national total to **6,250 confirmed cases and 3,039 deaths**.

• Ituri remains the epicenter of the epidemic, accounting for 81.7% of cumulative confirmed cases and 60.9% of new cases reported in the last 24 hours, followed by North Kivu province, which has the highest case fatality rate. Strengthening priority interventions (surveillance, care, infection prevention and control, logistics, and community engagement) remains essential to rapidly break the chains of transmission.

TIMELINE OF KEY EVENTS

• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

• Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a national and then continental public health emergency, accompanied by mobilization funds.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd **Incident**

Manager: Professor

Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. Kombe NGWAMA John

Editorial Team

Professor Dr. Mulangu Katulumba Félix-Médard

Dr. Katusi Biselenge Jules

Mrs. Mayele Afua Marie

Dr. Diur Mananashi Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd